



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

Guideline

Nursing staff escalation – Mental Health

Reference #: FSH-MENH-GUI-0005

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital (Incl Hampton House) Cockburn Health	Mental Health Service	Nursing

1. Introduction

This guideline provides a framework to guide nursing staff in the escalation of issues or concerns regarding mental health patient management. Issues may be clinical and/or operational. The escalation process informs nursing staff senior nurse/s availability to support the nursing team and applies to both mental health inpatients and patients managed in the community.

2. Terminology

Escalation	A protocol endorsed by the organisation to guide staff how to best seek direction from senior clinicians when there is concern regarding the clinical care of a patient or when managing patients whose physical or mental state is deteriorating.
iSoBAR	Mnemonic used as a tool to guide the content and structure of handover in a manner that suits the clinical context. Identify, Situation, Observations, Background, Agree plan, Read back.
Adult Observation and	An observation chart used for patients aged 16 years and over. It includes the Adult Deterioration Detection System (ADDS) with weighted clinical observations with parameters and triggers which

Response Chart (AORC)	indicate clinical deterioration. The ADDS aids clinicians to safely plan the next steps.
Hospital Out of Hours Team (HOOT)	Provide out of hours staff support for clinical and staffing issues including: • Change in acuity levels impacting staff skill mix • Absent or missing patient • Discharge against medical advice • S4/S8 drug discrepancy • SAC 1 and SAC 2 incidents • Inability to obtain specific medications • Restraint and/or seclusion events • Deteriorating or unpredictable behaviour of a community or inpatient patient e.g. thoughts or acts of self-harm / suicide • Any allegation made toward a staff member • Any allegation towards a patient (e.g. sexual/ physical assault, theft) • Infrastructure issues
Assessment Liaison Response Team (ALERT)	Provide support for patient safety issues: • Clinical deterioration as per ADDS • Inability or delay to get medical review
Mental State Deterioration	A change in a person's mood or thinking, marked by a change in behaviour, cognitive function, perception or emotional state indicating the patient requires escalated care.
PACE	Probe: "Can I CLARIFY....?" Alert: "Perhaps we should just check.....?" Challenge: "I am concerned about this patient.....I need your help?" Emergency: "For the safety of the patient you must STOP and listen"

3. Guideline

In situations where there is concern regarding the proposed management of any inpatient or community mental health patient, Nursing staff are authorised to directly contact the relevant Senior Nurse/ Clinician:

- NUM, CNS, CNC, Nurse Director/Nurse Coordinator in hours
- HOOT CNM / ALERT CNS after hours and/or
- Consultant on call.

Nursing staff may further escalate to the FSH SRN 7 Hospital Coordinator out of hours if unable to satisfactorily resolve concerns.

3.1. Escalation Triggers in care delivery

Inpatient mental health nursing staff will escalate issues/concerns including but not limited to:

- Inadequate staffing numbers to manage the ward e.g. staff member goes home sick mid shift.
- The skill mix on the ward for the shift.
- The number of patients and their level of overall acuity.
- Number of expected admissions and or discharges on shift.
- The number of patients on close observations and/or 1:1 Nursing special that makes nurse allocation difficult or unachievable.
- Planned escorts including escorted ground access and patient appointments.
- Ward layout impacting care
- Any other factors impacting on clinical activity or ward management including patient and staff safety.

3.2. Escalation process for patient care concerns

- Clinical handovers will be provided using the iSoBAR format.
- Patient care issues will be escalated using the PACE approach.
- The NUM and/or CNS are the point of contact between the hours 0730 and 15:30 hours, Monday – Friday.
- The HOOT CNM / ALERT is available from 14:30 to 08:30 hours Monday – Friday and weekends/Public Holidays are 24 hours.
- The treating/on call Consultant Psychiatrist must be contacted at any time of the day or night regarding unexpected deterioration in a patients condition.

- Refer to Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist
- The HOOT CNM is the after-hours point of contact for the following requirements:
 - Shift rounding and handover from inpatient staff
 - Patients of concern (inpatient or community)
 - Attendance at ALL codes within mental health wards
 - Nursing concerns regarding a staffing issue
 - Changing ward acuity impacting on staff skill mix
 - Nursing staff injury and/or sickness
 - Staff unable to complete shift
 - Infrastructure issues for example telecommunications/internet or application issue
 - Supply or equipment issue not resolved
 - Support with Schedule 4 and 8 medications
 - Workplace safety and health issues
- The CNM/HOOT is the point of contact for the following:
 - Nursing staff shift deficits
 - Staffing grievances related to agency staff.

3.3. Hampton House escalation process

- Nursing staff at Hampton House will escalate criteria as per below:

Hampton House escalation process		
Criteria	In Hours	Out of hours
Medical review/ medication prescribing	● Community Treatment Team (CTT) ● HH Clinical Nurse Consultant (CNC)	● Duty Medical Officer (DMO) Assessment & Treatment Team (ATT)
Mental health deterioration	● CTT Team Leader ● HH CNC	● HOOT Clinical Nurse Manager/ ALERT Clinical

	• Programme Manager	Nurse Specialist (SI) • On call Consultant • FSH SRN L7 Hospital Coordinator • Nurse Director On call
Aggression	• Contact Emergency Services (WAPOL)	• Contact Emergency Services (WAPOL)
Medical emergency	• Contact Emergency Service (Ambulance)	• Contact Emergency Service (Ambulance)

- Nursing staff will escalate as per [Appendix 2](#) Hampton House Escalation flowchart

3.4. Fiona Stanley Hospital Cockburn Health

- Nursing staff at FSH Cockburn health will escalate as per [Appendix 3](#).

4. Compliance/performance monitoring

- The S5 Nurse Director and Nurse Coordinator will evaluate compliance with this guideline via routine incident review processes.

5. Related Policy Documents

- WA Health:
 - [Recognising and Responding to Acute Deterioration Policy \(MP0086/18\)](#)
 - [Clinical Handover Policy \(MP0095/18\)](#)
 - [Mandatory reporting of notifiable incidents to the Chief Psychiatrist](#)
- FSFHG:
 - [Escalation process for Nursing and Midwifery Staff](#)

- Clinical Handover Policy
- Observation and Response Chart
- Emergency Procedures

6. Related standards

- NSQHS standard:
 - Clinical Governance
 - Communicating for Safety
 - Recognising and Responding to Acute Deterioration
- Chief Psychiatrists Standards for Clinical Care

7. Authorisation

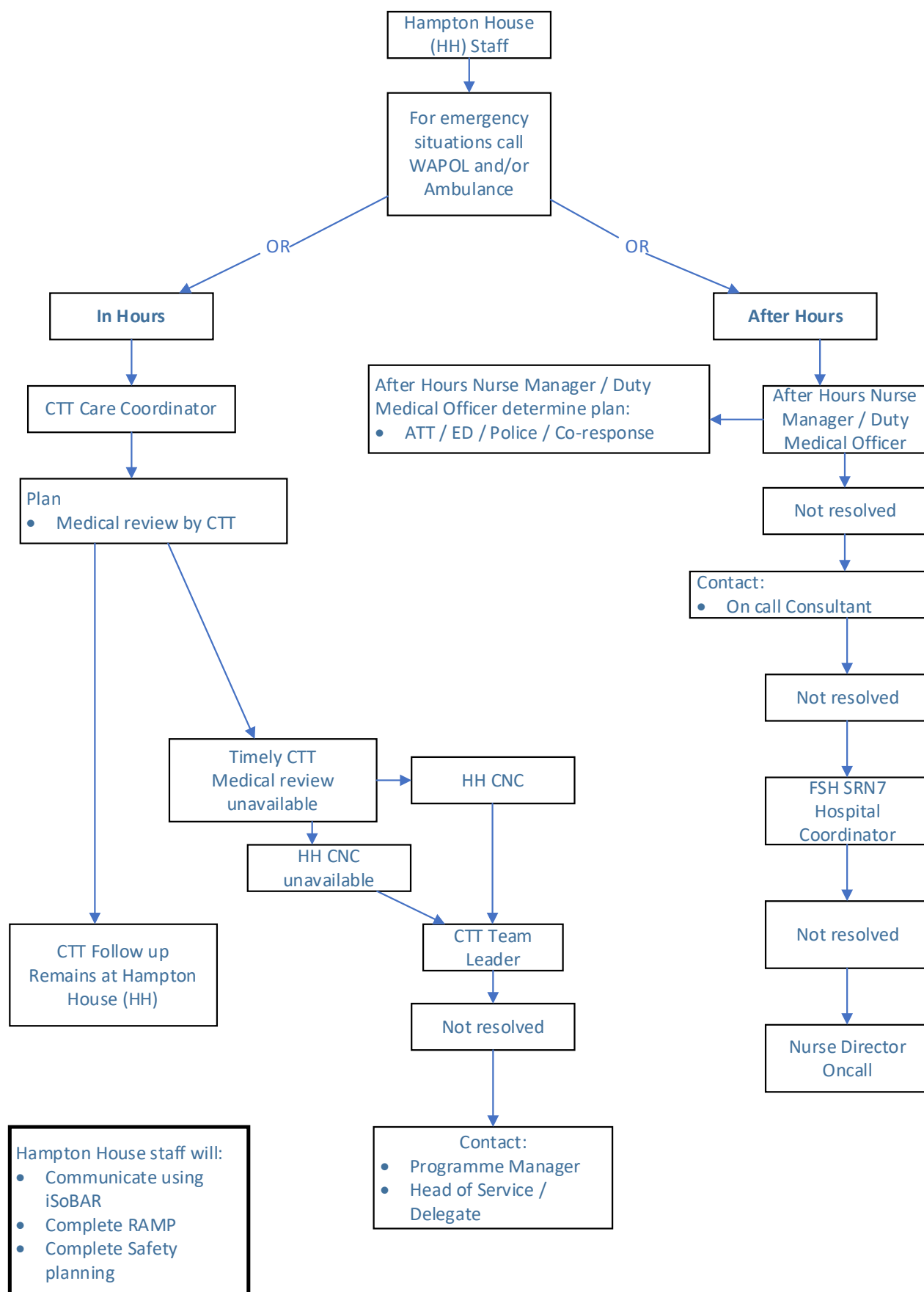
EXECUTIVE SPONSOR: Nurse Director-Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Authorised By	Revision due
1	05/2021	Nurse Director	S5 SQR Committee – Departmental policy	S5 SQR Committee (Single Speciality Process)	05/2023
1.1	05/2021	Nurse Director	S5 SQR Committee – Departmental policy	S5 SQR Committee (Single Speciality Process)	05/2023
2	08/2023	Nurse Director	NMPGC	S5 Safety Quality and Risk Committee	08/2025
3	08/2024	Nurse Director	Minor update no change to review date	S5 Safety Quality and Risk Committee	08/2025
4	10/2025	CNS – Quality improvement	S5 SQR Committee – Departmental policy	S5 Clinical Governance Committee	10/2027

8. Appendices

8.1. Appendix 1: FSH nursing staff escalation process – when no MH Clinical Nurse Manager on duty

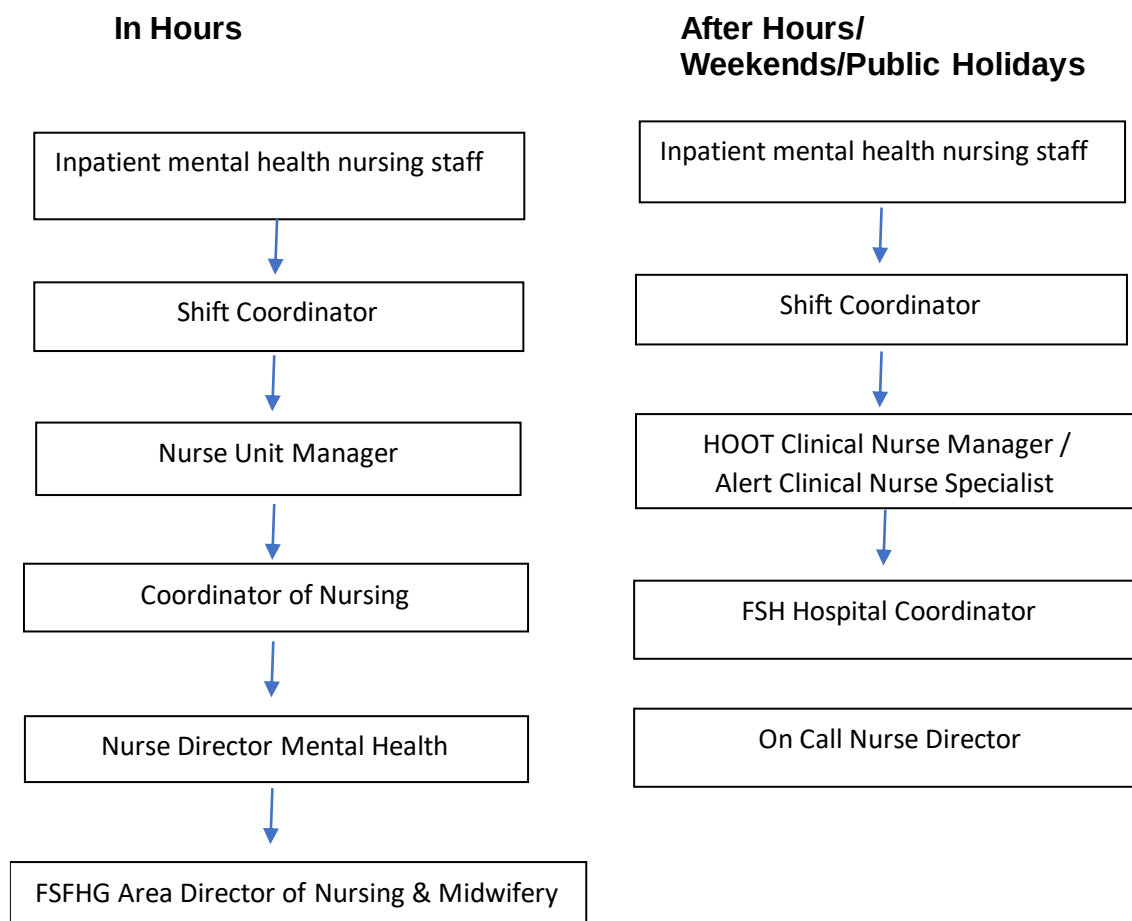


8.2. Appendix 2: Hampton House Escalation of Care



8.3. Appendix 3. FSH Cockburn Health Escalation of Care

Nursing issue/concern identified during your shift.
Follow the cascading escalation process for issue /concerns that require to be communicated to the senior nursing team .



In situations where there is concern, disagreement or conflict regarding the proposed management of any inpatient, Mental Health Nursing staff are authorised to directly contact the Senior Nurse / Consultant in hours or on call Consultant and HOOT CNM after hours).

Ref: Escalation process for Nursing and Midwifery staff. FSFH-HW-POL-0014